STIs ON LABOR AND DELIVERY: WHAT TO DO NOW

FD-ANT-STI · Companion to STIs on Labor and Delivery · CDC STI Treatment Guidelines 2021 · ACOG PB 220 · Labor and delivery / OBED · Decision support only, not unit policy

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Last reviewed: Provisional, Nov 2026

PROVISIONAL

HOW TO READ THIS CHART: start by choosing **In labor / admitted** or **OB ED triage**. The labor path is the router below (hard stops first). The OB ED triage branch is for patients not in labor or postpartum ≤12 wk. Doses and regimens live on the companion card; this chart lists none.

SHADING KEY: WHITE = can wait / no delivery-mode change · LIGHT GRAY = admission check · MID GRAY = treat before discharge · DARK GRAY = hard stop — changes delivery

START: IN LABOR / ADMITTED, OR OB ED TRIAGE?

Choose the path that matches how she presents.

IN LABOR / ADMITTED · WHITE
Continue to the labor router (nodes 1–5 below).

OB ED TRIAGE · WHITE
Not in labor (any GA) or postpartum ≤12 wk. Continue to the OB ED triage branch below.

1. ADMISSION STATUS CHECK · LIGHT GRAY
Syphilis, HIV and HBsAg must be known; also GC/CT and HSV history. Anything unknown gets ordered now. Then go to node 2.

2. HARD STOPS — SHOWN FIRST

Does anything change how she delivers? Check both boxes. Either or both may apply.

ACTIVE GENITAL HSV · DARK GRAY
Active genital HSV lesions or prodrome, or ROM with lesions: cesarean. This is a hard stop. See the companion card for the rest of HSV management.

HIV · DARK GRAY
HIV reactive or unknown with risk, or viral load over 1,000 or unknown: IV ZDV, then the HIV card / HHS guidance for mode of delivery. This is a hard stop.

3. TREAT BEFORE DISCHARGE — DELIVERY MODE UNCHANGED · MID GRAY
These do not change how she delivers. Treat before she leaves. Doses and regimens are on the companion card.

SYPHILIS · MID GRAY
Positive or undocumented, including discordant results: benzathine penicillin by stage. Desensitize if she is allergic. Peds handoff with her titers and treatment dates. (Doses on the card.)

GC OR CT · MID GRAY
Positive or untreated: treatment per the card. Tell peds about untreated GC. Order EPT.

4. CAN WAIT UNTIL POSTPARTUM OR ROUTINE CARE · WHITE
Trich, symptomatic BV, yeast, test of cure, and suppression counseling. **Fluconazole only after delivery.** Regimens are on the companion card.

5. PEDS HANDOFF SUMMARY · LIGHT GRAY
Auto-list whatever was flagged: syphilis; HSV (primary vs recurrent); untreated GC; HBsAg status; and HIV. Use the peds-handoff cue with this algorithm. Nothing is stored.

OB ED TRIAGE BRANCH · LIGHT GRAY

Not in labor (any GA) or postpartum ≤12 wk. Early pregnancy, location unknown, or pain/bleeding <20 wk: confirm pregnancy and location first; ectopic and septic abortion outrank every STI diagnosis.

UNIVERSAL FIRST STEP · LIGHT GRAY
GA/viability and FHT or EFM. Vitals and sepsis screen. Last prenatal labs (syphilis, HIV, HBsAg, GC/CT, urine culture) — draw any that are missing. Ask about partners, prior STI, and social risk. Contractions, bleeding, ROM or abnormal FHR → L&D;, not this pathway.

a) DISCHARGE / ITCHING · WHITE
Workup and treatment per the card. Trich: always treat, including an incidental asymptomatic finding. BV symptomatic only. VVC: no oral fluconazole in pregnancy.

b) LESIONS / ULCERS · WHITE
HSV PCR, syphilis serology, HIV. Darkfield/PCR if available (do not wait). Pending house decision on darkfield/PCR availability. Treatment per the card.

c) PELVIC PAIN / FEVER · WHITE
Rule out ectopic, septic abortion, IAI, pyelo first. PID in pregnancy: hospitalize, IV, ID consult. Regimen per ID/pharmacy. Pending house decision.

d) DYSURIA · WHITE
UA, culture, and GC/CT/trich NAAT. Pyelonephritis: admit as an inpatient for IV antibiotics. Cystitis/ASB: treatment per the card.

e) SYPHILIS SCREEN · WHITE
Treat in OBED; do not send her home for the shot. House practice (not a cited rule): J-H — before ~24 wk, discharge with return precautions; from 24 wk, first early-syphilis dose on L&D; with fetal monitoring for several hours.

f) PARTNER / EPT g) HIV OR HEP B UNKNOWN · WHITE
Test and treat exposures per the card. EPT permitted in Texas for GC/CT, not syphilis. Unknown HIV: opt-out test, link to care. Unknown hep B: SMFM #69 — newborn vaccine plus HBIG within 12 h.

BEFORE DISCHARGE · LIGHT GRAY
Single-dose treatments given in OBED, not just prescribed. Doses live only on the companion card. TOC/retest, Texas reporting, partner treatment or DIS, prenatal follow-up within 1 wk. Return precautions.

PROVISIONAL — pending Michael review and CSM pharmacy check. Not live. Doses and regimens live only on the companion card (STIs on Labor and Delivery). The OB ED triage branch is for patients not in labor or postpartum ≤12 wk. Decision support only, not orders or unit policy. Nothing typed into the cues is saved.